

Q I had an abortion when I was younger. Can I still get pregnant?

If your abortion was conducted before 14 weeks of pregnancy, and as long as your fallopian tubes were not damaged for any reason, you should still be able to get pregnant.

If your abortion was between 14 and 24 weeks, there may be a slight risk of premature delivery if you become pregnant again. If you have been trying to conceive for a year or more, or for six months if you are over 35, you may want to see your doctor to have some basic fertility tests.

Q I'm on long-term medication. Will this affect my fertility?

Some medication prescribed for long-term medical conditions may affect your ability to conceive. If you have an existing medical condition, it's important to talk to your doctor before you try to get pregnant.

Such conditions include asthma, diabetes, heart disease, inflammatory bowel disease (IBD) lupus, epilepsy, migraine, mental health problems, acne, high blood pressure, anemia, obesity, and being underweight. The type or dose of prescription drugs, vitamins, herbal medicine, or other supplements you take may need to be adjusted before and during pregnancy. Don't stop taking any medication you've been prescribed until advised to do so by your doctor. Don't forget to mention any allergies and over-the-counter medications too.

You may need to take a higher dose of folic acid if you are at risk of having a pregnancy affected by a neural tube defect (see p.49). This may be the case if you take medication for epilepsy, you are diabetic, if you had a neural tube defect in a previous pregnancy, or there is a history of neural tube defects in your family.

Q I think my partner and I have fertility issues. What should we do?

The first thing to do is make an appointment with your doctor, especially if the female partner has irregular or painful periods, feels pain during intercourse, has previously had a pelvic infection or surgery, or if there's any reason the male partner might have abnormal

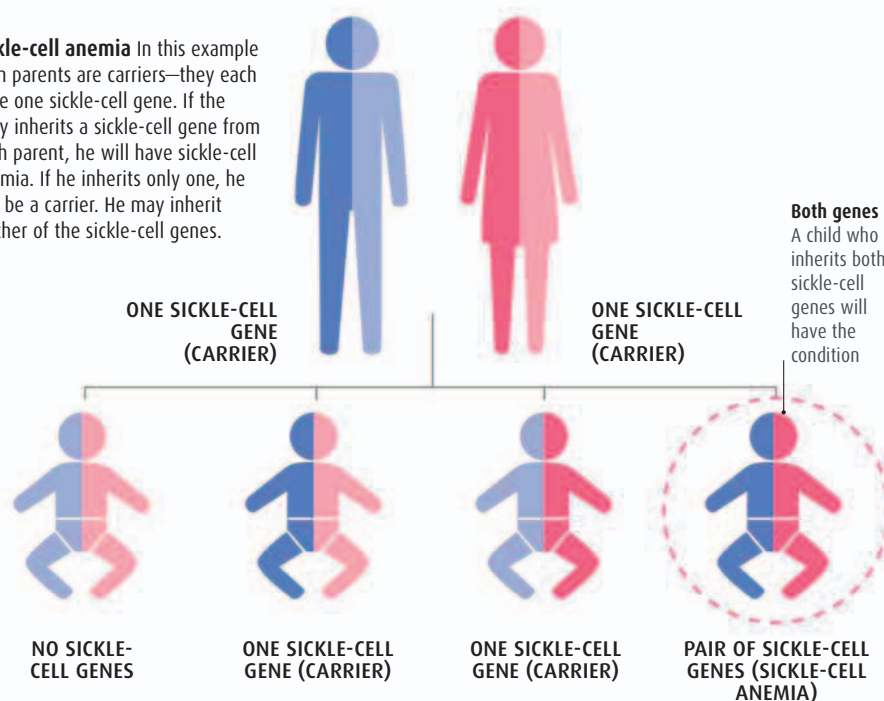
Q What is genetic screening and should we have it before we conceive?

Genetic screening may be done to find out if couples who are planning to conceive are carrying a particular altered gene that could lead to their baby inheriting a specific medical condition. It helps couples to prepare for the likelihood of their child having the condition.

Some genetic disorders are more common in particular ethnic groups. Sickle-cell disease is most common in those of African descent, while most cases of Tay-Sachs disease and cystic fibrosis affect Ashkenazi Jews and northern white Europeans respectively. Contact your doctor to be referred for

screening. The test usually involves giving a blood or sample tissue so that your DNA can be analyzed. It may take weeks, or sometimes months, to get the results. It is vital to remember that screening is not foolproof or conclusive. Genetic counseling is often provided to inform and support parents.

Sickle-cell anemia In this example both parents are carriers—they each have one sickle-cell gene. If the baby inherits a sickle-cell gene from each parent, he will have sickle-cell anemia. If he inherits only one, he will be a carrier. He may inherit neither of the sickle-cell genes.



semen. Your doctor will ask you how long you have been trying to conceive, the current state of your health, and about any past health issues. If appropriate, he or she will suggest a range of tests that may establish the cause.

For women, the possible causes include damaged or blocked fallopian tubes, failure to ovulate, endometriosis, fibroids, cysts on the ovaries, and pelvic inflammatory disease. The most common tests are blood tests to check levels of hormones linked to ovulation, ultrasound scans to look at the reproductive organs, and X-rays of the uterus and fallopian tubes. For men, the causes include poor sperm count, blockage of the tubes, and infections.

Tests will assess sperm quality, hormones, the reproductive organs, and chromosomes.

Depending on the situation, there are various treatment options such as medicines and drugs to assist fertility, surgical procedures, and assisted conception. Although the tests may discover the root of the problem there are occasions where fertility can be unexplained.

Going through these tests can be extremely stressful and frustrating. During this time, it is vital to learn everything you can about your medical situation, so ask your doctor or specialist if you have any questions or concerns, and communicate with your partner about your feelings.